NCCP 00361 · VERSION 4

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Make treatment fit into daily life

Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself.

The first month is different

There is an extra injection visit two weeks after the first dose. After the Day 29 dose, visits continue every 28 days. Ask the team to confirm each appointment.

Before an injection

Tell staff about anticoagulants, unusual bleeding or bruising. Do not stop an anticoagulant yourself. The team reviews injection safety and any dose modification.

Injection technique and aftercare

Staff give deep intramuscular gluteal injections slowly. Report severe pain, numbness or pain radiating down the leg promptly.

If a visit is delayed

Contact the clinic to agree the next date. Do not calculate a catch-up or extra dose from this reference graphic; dose scheduling remains the clinical team’s responsibility.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00361 v4 · official regimen
- EMA Faslodex product information
- CONFIRM: final overall survival, fulvestrant 500 mg versus 250 mg
- FALCON: fulvestrant versus anastrozole in endocrine-naive advanced breast cancer
- FALCON: prespecified final overall survival
- Pan-Asian adapted ESMO early breast cancer guidelines
- Pan-Asian adapted ESMO metastatic breast cancer guidelines
- NICE NG 101: early and locally advanced breast cancer
- ESO-ESMO ABC 5 consensus: tumour-marker interpretation

Source reconciled; independent clinical, pharmacy and patient/PPI review pending